Committee: World Health Organization (WHO)

Topic: The Question of Combating Social Exclusion and Employment Discrimination Against Individuals with Medical Conditions

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Introduction

The relationship between health and employment is central to why this issue demands international attention. For individuals without a medical condition, employment is generally stable and predictable. For individuals with a medical condition, however, the ability to obtain and keep a job often depends on factors beyond their control, such as employers' attitudes, workplace flexibility, and whether national laws are actually enforced. Exclusion from work not only causes financial hardship; it also worsens physical and mental health, creating a cycle in which illness leads to job loss and job loss leads to further decline in health. For this reason, the World Health Organization regards employment inequality not simply as an economic matter, but as a matter of health equity.

Each medical condition creates its own distinct form of exclusion. Cancer survivors represent a growing population as treatment outcomes improve, and a systematic review estimated that the overall rate of return to work among cancer patients was about 72%. Yet returning to work does not always mean returning to equal treatment, as many survivors face reduced hours and limited promotion opportunities. People living with HIV face a different barrier. Although modern treatment allows them to work productively for decades, stigma remains widespread. The International Labour Organization's global survey revealed that stigmatizing and discriminatory attitudes are fuelled by a lack of knowledge on HIV transmission, and that as many as six in ten people supported mandatory HIV testing before people are allowed to work. Individuals with mental health conditions and invisible disabilities, such as autism or ADHD, frequently conceal their conditions out of fear of being labeled unreliable, which prevents them from requesting any support at all.

Experimental research has confirmed that this exclusion is driven by discrimination rather than by a lack of ability. In a Swedish field experiment, nondisabled applicants received 33% more callbacks than similarly qualified wheelchair users despite applying for jobs for which the impairment should not interfere with performance. A similar study in the United States found that applications disclosing a disability received 26 percent fewer expressions of employer interest, even though the disabilities were chosen because they would not be expected to limit productivity in accounting positions. Furthermore, the disadvantage becomes more severe when disability overlaps with other social factors such as gender, race, and income, meaning that certain groups experience multiple layers of exclusion at the same time.

Although international frameworks such as the Convention on the Rights of Persons with Disabilities (CRPD) have established the right to work on an equal basis, a significant gap remains between these commitments and the reality faced by millions of individuals. Policies designed mainly for visible, physical disabilities often fail to protect people with chronic, episodic, stigmatized, or invisible conditions. Therefore, this committee is expected to develop solutions that recognize the diverse nature of medical conditions and that address not only hiring, but also job retention, workplace inclusion, and full participation in society.

During the past few decades, job insecurity has become a critical problem in global society. Whether their conditions are visible or not, people with physical or psychological medical conditions have continuously faced discrimination in employment. They experience various types of exclusion, including alienation in the workplace and limited participation in their communities. This phenomenon did not occur by coincidence; it is the result of several social conditions.

It is necessary to clearly understand what this question addresses and why it has a broader meaning than the general concept of “disability.” The term “medical condition” includes conditions that are already recognized in national disability laws, as well as conditions that are chronic or episodic. Infectious diseases such as HIV and AIDS, and mental health conditions such as depression or schizophrenia, are also notable examples of medical conditions that can lead to social exclusion. Because each of these conditions creates a different kind of exclusion, they cannot be addressed by a single policy.

Definition of Key Terms

Medical Condition

Medical Condition refers to the physical, mental, and psychological state of human health. Whether it is temporary or permanent, it has the capability to interfere with the social participation of an individual. This can affect one's employment and social interactions within the community. The term has a broader meaning compared to “disability” since it contains all chronic and episodic conditions. Impairments, diabetes, and HIV/AIDS are the most notable examples of these health conditions. This term would act as a framework for identifying problems of various forms of discrimination related to physical and mental health conditions.

Disability

According to the UN Convention on the Rights of Persons with Disabilities (CRPD), disability results from the interaction between a person’s long-term impairment and the barriers in their environment that hinder their full and equal participation in society. This means that disability is not limited to a physical injury or condition, but also includes the barriers that prevent people from engaging in social activities. For instance, a wheelchair user is disabled not only by their impairment but also by staircases, narrow doors, and buses without wheelchair lifts or ramps. The common understanding of disability is often one-dimensional; disability should instead be understood in terms of society’s responsibility to remove these systemic obstacles.

Chronic Illness

Chronic illness is a health condition, whether it is physical or psychological, that persists for a very long period of time, approximately three months to a year, or longer. Other terms like chronic disease, chronic condition, or chronic disorder all roughly refer to the same thing: these are mostly incurable and lifelong. Obesity, high blood pressure, high cholesterol, cancer, and asthma are common examples of this condition. Having a chronic condition might mean that one needs regular ongoing medical care. These kinds of illnesses can frequently interfere with people's daily activities, causing excessive stress. For example, using the toilet or eating by oneself might not be as easy as expected.

When people are unable to carry out basic daily activities, their quality of life can be seriously affected.

Social Exclusion

Social Exclusion refers to the phenomenon when individuals or certain groups are marginalized and excluded from society. This consists of systematic alienation and prohibited access to essential resources, opportunities, and social networks. This process is currently a major concern as it threatens the citizens' well-being, social cooperation, and equality. Discussing social exclusion requires an understanding of the root causes and implementing practical policies.

Employment Discrimination

Employment Discrimination refers to the unfair treatment of individuals in employment-related decisions, such as hiring and promotion, based on factors unrelated to the job, such as race, age, gender, religion, nationality, or disability. The exact definition of employment discrimination, however, varies across countries and international agreements. For instance, ILO Convention No. 111 (1958) lists race, colour, sex, religion, political opinion, national extraction, and social origin as grounds of discrimination, but does not explicitly include disability or health status, although it allows member states to add other grounds. In contrast, the CRPD (2006) defines discrimination on the basis of disability to include the denial of reasonable accommodation. This difference shows why individuals with medical conditions are not always fully protected under existing frameworks.

At the national level, many countries have adopted their own laws. For example, in the United States, the Civil Rights Act of 1964 prohibits discrimination based on race, colour, religion, sex, and national origin, while the Americans with Disabilities Act (ADA) of 1990 separately prohibits discrimination against people with disabilities in job applications, hiring, wages, and promotions.

Reasonable Accommodation

A reasonable accommodation is an adjustment to a work environment or job tasks that allows a qualified worker to substantially perform the job in question. This may include workers who require reasonable accommodation to exercise their religion. More commonly, it includes reasonable

accommodations to enable workers with disabilities to perform the job. The Americans with Disabilities Act (ADA) prohibits employers and other covered entities from discriminating against workers with disabilities by failing to make reasonable accommodation to enable the worker to substantially perform their task (unless the employer or covered entity can show undue hardship)

Stigma

Stigma refers to the negative attitudes, beliefs, and stereotypes that society attaches to a particular group of people, which often lead others to view them as different or less valuable. In the context of employment, stigma can cause employers and colleagues to believe that a person with a medical condition is less capable, less reliable, or even dangerous, even when the condition does not affect their ability to work. For example, people living with HIV may face stigma because of misconceptions about how the virus is transmitted, and people with mental health conditions may be labeled as unstable. Stigma is especially harmful because it often leads individuals to hide their conditions, which prevents them from requesting the support they need.

Employment Quota

An employment quota is a legal requirement that obliges employers, usually those above a certain size, to ensure that persons with disabilities make up a set percentage of their workforce. Employers that do not meet the required percentage are often required to pay a fine or levy, which is commonly used to fund disability employment programs. Countries such as France, Germany, and Japan use quota systems, while others, such as the United Kingdom and Sweden, rely mainly on anti-discrimination laws and support programs instead. Supporters argue that quotas guarantee a minimum level of employment, while critics argue that some employers hire only enough workers to avoid penalties without ensuring genuine inclusion.

Background Information

Different Medical Conditions Creating Different Kinds of Exclusion

Although all individuals with medical conditions may face exclusion in the workplace, the causes and forms of this exclusion differ greatly depending on the type of condition. Cancer survivors are one clear example. In recent years, the number of people who have survived cancer has been increasing noticeably, and most of them are eager to return to work or apply for a new job. However, their return to employment is not guaranteed. A systematic review estimated the overall return-to-work rate among cancer patients at around 72%. Although this rate may seem high, it varies depending on the type of cancer, the side effects of treatment, and the support available after treatment. Moreover, even when survivors return to their jobs, many experience reduced working hours, difficulties being promoted, or discrimination. A long-term study in France found that, even with labor protection laws and health insurance systems in place, experiencing workplace discrimination increased survivors' probability of job loss by 15%. Another study in the United States found that survivors of childhood cancer were about three times more likely to be unemployed than their peers, and that survivors from low-income backgrounds were less likely to work in jobs that offer flexible schedules or sick leave.

People living with HIV face a different form of exclusion. Thanks to advances in medical treatment, most people living with HIV are fully capable of working productively for many years. However, their social exclusion remains an international issue, mainly because of misconceptions and prejudice. A global survey conducted by the International Labour Organization (ILO) and Gallup, published in 2021, collected responses from more than 55,000 people in 50 countries. It found that in certain regions, a large share of respondents were unwilling to work alongside people living with HIV. In Asia and the Pacific, only about 40% of respondents agreed that people living with HIV should be allowed to work directly with people who do not have HIV, and the figure was similarly low in the Middle East and North Africa, at about 42%. The survey also showed that these attitudes are fuelled by a lack of knowledge about how HIV is transmitted.

Because different medical conditions create different kinds of barriers, the policies that most nations currently apply are not sufficient to solve the issues mentioned above. For instance, while a wheelchair ramp helps people with mobility impairments, it does nothing for a person who hides a mental health condition out of fear of being dismissed. Similarly, laws that ban discrimination against people living with HIV do not help cancer survivors who face discrimination while adjusting to their

workplace after treatment. If no action is taken, these issues are likely to lead to social isolation, poorer mental health, and a weakened sense of belonging in society.

Health, Illness, and the Right to Work

An individual's health extends beyond basic medical diagnosis; this status has the ability to indicate one's future expectations. Representative examples of these expectations include whether the person is employable, whether they can secure their place in the company, and whether they are capable of adapting to the work environment and improving their working skills. While employees without chronic or mental health conditions are generally hired and rarely face such problems in their workplaces, people with medical conditions often have less stable working lives. They frequently experience different types of discrimination at work. This discrimination includes not only unfair treatment by employers and colleagues but also structural problems, such as workplace facilities that are not accessible to people with medical conditions. These individuals are often excluded from opportunities and team projects, which can lower their motivation and harm their mental health. Limited promotions, unequal employment opportunities, and difficulty participating in collaborative work are well-known examples of the discrimination that contributes to poorer mental health among people with disabilities.

Resume Audit Studies

Recently, statistics show the existence of employment gaps between people with disabilities and those who do not have them. There are still several questions remaining about these gaps: is it because of discrimination, or the difference in performance? Researchers have used a method called "resume audit study" to further investigate the exact cause. They created two separate resumes which are mostly similar to one another – similar skills, job experience, and their strengths – except for their health conditions. Then they sent those resumes to various businesses to calculate the percentage of being called back. Since those two resumes were almost exactly the same, the gap in the number of callbacks was mainly caused by medical conditions, not people's abilities.

The results of these experiments were consistently unfavorable. In Sweden, researchers conducted a large field experiment with real employers and found that applicants without disabilities

received 33% more callbacks than equally qualified wheelchair users. This finding is especially significant because the same gap appeared even for jobs in which using a wheelchair would not affect job performance. Another study in the United States, which used fictitious resumes, found that simply disclosing a disability reduced employer interest by 26% compared to applicants who did not mention any health condition. Furthermore, a large meta-analysis of hiring experiments found that the average callback gap for applicants with physical disabilities was larger than the average gap found in studies on race and national origin. This suggests that hiring discrimination against people with physical disabilities may be even more severe than discrimination based on race or national origin.

Disabilities Combining with Social Factors

Disability does not exist in isolation from other social factors. When disability is combined with other forms of disadvantage, people can face compounded barriers. For instance, numerous studies show gaps even between men and women with disabilities: women with disabilities tend to have lower employment rates, fewer promotions, lower labor force participation, and lower salaries than men with disabilities. This case shows that disability can deepen the disadvantages faced by certain marginalized groups when it is combined with other forms of inequality.

The issue becomes even more complex when race is considered. One study found that white job applicants received about 53% more callbacks and 145% more job offers than applicants from racial and ethnic minority backgrounds. When disability is added to this situation, the gap is likely to widen further. A large study in the United States found that disabled Black men and disabled Black and American Indian or Alaska Native women in middle adulthood had the highest predicted probability of unemployment among all groups studied.

Current State of Invisible Disabilities

Most nations' disability measures are designed mainly for people with visible, physical disabilities, such as ramps, accessible restrooms, and reserved parking spaces. However, recent studies show that the number of people identified with invisible disabilities is growing rapidly. These include neurodevelopmental conditions such as Attention-Deficit/Hyperactivity Disorder (ADHD) and Autism Spectrum Disorder (ASD). Neurodevelopmental conditions are differences in how the brain

develops, which affect how a person communicates, focuses, processes information, and interacts with others. They are different from mental health conditions such as depression: they are usually present from early childhood and continue throughout a person's life. Because these conditions are invisible, many employers misunderstand autistic employees and employees with ADHD, or treat them unfairly.

Employment data for autistic adults vary from country to country, but they all point to a similar conclusion. In the United Kingdom, official government data show that only 34.0% of disabled people with autism were employed in the 2024/25 financial year, compared to 55.3% of all disabled people and 82.0% of non-disabled people. A high level of education does not fully solve this problem. According to the UK government's Buckland Review of Autism Employment (2024), autistic graduates are twice as likely as non-disabled graduates to be unemployed 15 months after graduation, and only 36% find full-time work.

Possible solutions

Managing Diverse Types of Accommodation Standards that Specifically Target Medical Conditions

Medical conditions such as chronic illnesses, mental health conditions, HIV, and fluctuating conditions such as asthma and atopic dermatitis can create barriers to equal participation in the workplace. Despite the efforts of many nations to promote reasonable accommodation, these policies often fail to fulfill their role because most of them focus on visible disabilities, such as those of wheelchair users. However, the scope of this issue is much broader, and nations should recognize the different kinds of medical conditions, since each creates its own challenges at work.

For wheelchair users, the main obstacles may be buildings with many staircases, narrow doors, and a lack of accessible restrooms. For people living with HIV, the main barrier is rarely their ability to work, since most can work productively with modern treatment; instead, their main concern is stigma and the fear of being treated unfairly or dismissed if their HIV status becomes known. People with mental health conditions often hide their conditions out of fear of being seen as unreliable, and as a result, they may not receive any support from their employers or colleagues. Lastly, for people with fluctuating conditions, such as Parkinson's disease, asthma, and diabetes, the challenge is that their symptoms can change from day to day. Their symptoms may suddenly appear on an important day, even if they had little effect on other days, and many businesses prefer to avoid this uncertainty. Because each of these problems is distinct, nations should introduce policies that target specific categories of medical conditions.

Establishing Shared Accommodation Funds to Support Employers, Especially Small Businesses

Many nations, including France, Germany, China, and Japan, rely on employment quota systems that require companies to hire a certain percentage of people with disabilities or pay a fine. While these systems increase the number of workers hired, they do not always guarantee genuine inclusion. Some companies hire only enough workers to avoid penalties, without providing real responsibilities, promotion opportunities, or the accommodations these workers need. One major

reason for this problem is cost. Large corporations can usually afford ramps, assistive technology, or flexible working arrangements, but small and medium-sized enterprises often cannot, even when their owners are willing to hire people with medical conditions.

To address this issue, member states could establish a national or regional accommodation fund. This fund could be financed by the government, by fines collected from companies that fail to meet quota requirements, or by a small contribution from all employers. When a company needs to install accessible equipment, arrange a phased return-to-work program for a cancer survivor, or provide flexible schedules for an employee with a fluctuating illness, it could apply to this fund to cover part or all of the costs. Germany's Integrationsämter already provides a similar model by offering financial and technical support to employers. Expanding this approach internationally would reduce the financial burden on employers, encourage long-term employment rather than short-term hiring to avoid fines, and allow nations with limited budgets to share resources through regional cooperation. The World Health Organization, together with the International Labour Organization, could provide technical guidance to help member states design and monitor such funds.

Integrating Employment Support Into Healthcare Systems

In most nations, healthcare and employment are handled by separate ministries and institutions that rarely communicate with each other. As a result, a patient may receive excellent medical treatment for cancer, depression, or a chronic illness, but still lose their job during treatment because no one helped them plan their return to work. This gap is especially harmful because unemployment itself worsens health, creating a cycle that is difficult to escape.

Member states could address this problem by including employment support as a standard part of medical treatment. For example, when a patient is diagnosed with a serious or long-term condition, the treatment plan could include access to a vocational counselor. This counselor would help the patient communicate with their employer, plan a gradual return to work instead of an immediate full-time return, and understand their legal rights so that they can recognize and report unfair treatment. Hospitals and clinics could also provide medical documentation that clearly explains which workplace adjustments a patient needs, without revealing sensitive details such as the specific diagnosis. This would protect the privacy of people with stigmatized conditions such as HIV or mental illness, while still allowing them to receive support. The World Health Organization could

encourage this approach by issuing official guidelines that recommend formal cooperation between health ministries and labour ministries, and by collecting data from member states on return-to-work outcomes. In this way, keeping a patient's job would become a recognized part of their overall care, rather than a burden they must face alone.

Strengthening the Enforcement of Anti-discrimination Laws

Many nations have already adopted laws that prohibit employment discrimination against persons with disabilities or medical conditions. However, having a law is not the same as enforcing it. In many countries, complaint procedures are complicated, legal support is expensive, and individuals fear losing their jobs if they report discrimination. As a result, a large amount of discrimination is never reported or punished. In addition, laws often do not reach the informal sector, where many persons with disabilities work, and the main international convention on employment discrimination, ILO Convention No. 111, does not list disability or health status as a ground of discrimination.

To address this issue, member states could establish or strengthen independent national bodies that can receive complaints, investigate cases, and impose meaningful penalties on employers that discriminate. Complaint procedures should be simple, confidential, and free of charge, and individuals should have access to free legal advice. Member states could also require larger employers to report data on the employment, retention, and promotion of persons with disabilities, so that progress can be measured and compared over time. Furthermore, member states could use Article 1(1)(b) of ILO Convention No. 111 to formally add disability and health status as protected grounds in their national laws. The World Health Organization, together with the International Labour Organization, could develop common indicators for monitoring employment discrimination and provide technical support to nations with limited enforcement capacity.

Promoting Anti-Stigma Education in Workplaces and Communities

As discussed earlier in this report, a major cause of employment discrimination is not the medical condition itself, but the misunderstanding and fear surrounding it. The ILO and Gallup survey found that stigmatizing attitudes toward people living with HIV are largely fuelled by a lack of basic knowledge about how HIV is transmitted. The same survey showed that 68% of people with tertiary

education agreed that people living with HIV should be allowed to work directly with others, compared to only about 40% of those with only primary education. This suggests that education can play a powerful role in reducing stigma.

Member states could introduce mandatory training programs for employers, human resources staff, and managers that cover basic facts about common medical conditions, the rights of workers with medical conditions, how to provide reasonable accommodation, and the importance of keeping health information confidential. These programs would be more effective if they involve persons who have lived experience of these conditions, since direct contact helps replace stereotypes with real understanding. At the national level, governments could run public awareness campaigns, similar to the United Kingdom's "Time to Change" mental health campaign, and include accurate information about disability, HIV, and mental health in school curricula. The World Health Organization could support this effort by developing standardized training materials and toolkits that member states can adapt to their own languages and cultural contexts.

Major parties involved

People's Republic of China

China manages the employment of persons with disabilities mainly through a national quota system, which requires employers to hire persons with disabilities for at least 1.5% of their workforce. Employers that fail to meet this requirement must pay into an Employment Security Fund, which is then used to support disability employment programs. With around 85 million persons with disabilities, China has one of the largest disability populations in the world, but access to accessible workplaces still differs greatly between wealthy eastern cities and rural areas. In committee, China is likely to support state-led solutions such as quotas and government funds, while emphasizing that each nation should be free to design policies suited to its own development level.

Russian Federation

Russia's Federal Law on the Social Protection of Persons with Disabilities sets regional employment quotas of about 2% to 4% for larger employers, and Russia ratified the CRPD in 2012. Russia has also expanded vocational rehabilitation programs in recent years. However, a gap remains between the quotas set in law and actual hiring practices, especially in regions far from major cities where accessible infrastructure is limited. In committee, Russia is likely to support quota-based approaches and national sovereignty in policy design, and to be cautious about binding international monitoring.

United Kingdom of Great Britain and Northern Ireland

The United Kingdom does not use an employment quota. Instead, the Equality Act 2010 prohibits discrimination based on disability, including long-term health conditions, and requires employers to make "reasonable adjustments." The government also runs the Access to Work program, which helps pay for workplace adjustments, and the voluntary Disability Confident scheme, which encourages inclusive hiring. The UK also collects some of the most detailed disability employment data in the world. In committee, the UK is likely to favor anti-discrimination laws, reasonable adjustments, and financial support over mandatory quotas, and may support the creation of accommodation funds similar to Access to Work.

French Republic

France is one of the strongest supporters of employment quotas. Companies with 20 or more employees must ensure that persons with disabilities make up at least 6% of their workforce, and employers that do not meet this requirement must pay contributions into national disability employment funds. These funds are then used to support training, workplace adaptation, and job placement for persons with disabilities. In committee, France is likely to defend strict quota systems and to support linking quota penalties to accommodation funds.

Federal Republic of Germany

Germany requires companies with 20 or more employees to fill at least 5% of their positions with persons with severe disabilities, and employers that fall short must pay a compensatory levy. This levy funds the Integrationsämter (integration offices), which provide financial and technical support to employers for workplace adaptation and job retention. Germany's system shows how quotas and support funds can work together. In committee, Germany is likely to promote its model as a balanced solution and to support international guidelines for accommodation funds.

Kingdom of Sweden

Sweden does not rely on quotas. Instead, it supports persons with disabilities through its broader welfare system, including sick leave, strong job protection, and wage subsidies for employers. Sweden also operates Samhall, a state-owned company that employs persons with disabilities and helps them move into the regular labor market. Sweden is often cited as having a relatively small disability employment gap among high-income nations. In committee, Sweden is likely to argue that comprehensive welfare and support systems are more effective than quotas, and to emphasize gender equality within disability policy.

Japan

Japan uses a mandatory employment quota under the Act on the Promotion of Employment of Persons with Disabilities. The required rate for private companies rose from 2.5% to 2.7% in July 2026, and employers that fall short must pay a levy, while those that exceed the rate receive financial support. Japan is also experiencing rapid population aging, and as a result, the number of workers

living with chronic illnesses has increased. In committee, Japan is likely to support quota systems combined with financial incentives, and to raise the issue of chronic illness among older workers.

Republic of India

India's Rights of Persons with Disabilities Act 2016 recognizes 21 types of disabilities, one of the broadest legal definitions in the world, and reserves 4% of government jobs for persons with disabilities. However, most Indian workers, including many persons with disabilities, work in the informal sector, where quotas and anti-discrimination laws are difficult to enforce. In the committee, India is likely to represent the concerns of developing nations, arguing that solutions must address the informal sector and that wealthier nations should provide financial and technical support.

Federative Republic of Brazil

Brazil's Quota Law (Law No. 8.213/1991, known as "Lei de Cotas") requires companies with 100 or more employees to fill 2% to 5% of their positions with persons with disabilities, depending on company size. Brazil is often discussed as an example of a nation trying to enforce quotas while dealing with a large informal economy and major regional inequality. In committee, Brazil is likely to support quota systems and to call for stronger cooperation among developing nations.

United States of America

The United States is home to the Americans with Disabilities Act (ADA) of 1990, one of the most influential disability anti-discrimination laws in the world, which requires employers to provide reasonable accommodation. However, although the United States signed the CRPD in 2009, it has never ratified it. In December 2012, the Senate voted 61 to 38 in favor of ratification, which fell short of the two-thirds majority required. In addition, the United States completed its withdrawal from WHO in January 2026, so it is not currently a WHO member state. Even so, the ADA continues to serve as an important reference point for anti-discrimination and reasonable accommodation frameworks around the world.

Republic of South Africa

South Africa's Employment Equity Act explicitly prohibits unfair discrimination based on HIV status as well as disability, and it restricts employers from requiring HIV testing of employees. In the landmark case Hoffmann v. South African Airways (2000), the Constitutional Court ruled that refusing to hire a person because of their HIV-positive status was unconstitutional. As one of the nations most affected by HIV, South Africa is a key voice on stigma-based discrimination. In committee, South Africa is likely to push for strong protections against HIV-related discrimination and for including health status as a protected ground in international standards.

Federal Republic of Nigeria

Nigeria's Discrimination Against Persons with Disabilities (Prohibition) Act 2018 prohibits discrimination based on disability and requires all public organizations to reserve at least 5% of employment opportunities for persons with disabilities. The law also gave a five-year transition period for making public buildings and transport accessible. However, implementation has been slow, and many persons with disabilities still face exclusion. In committee, Nigeria is likely to represent West African concerns, emphasizing the need for financial support and capacity building to enforce existing laws.

Arab Republic of Egypt

Egypt's Law No. 10 of 2018 on the Rights of Persons with Disabilities requires government and non-government employers with 20 or more workers to ensure that persons with disabilities make up 5% of their workforce, and offers tax benefits to employers who exceed this quota. Egypt also represents both the Arab region and North Africa. In committee, Egypt is likely to support quota systems combined with incentives, while emphasizing respect for national and cultural contexts in policy design.

Timeline Of Events

Date	Description of Event
UN Declaration on the Rights of Disabled Persons, 1975	The United Nations General Assembly begins to recognize the basic human rights of people with medical conditions, by ensuring the right of being employed.
ILO Convention No. 159, 1983	The ILO requires member states to enact international policies about vocational rehabilitation and employment, especially for people who are having disabilities.
UN General Assembly Declaration of Commitment on HIV/AIDS, 27 June 2001	The United Nations General Assembly adopts the first ever international commitment in order to support people who experienced stigma and discrimination caused by misconceptions of HIV, including the problems in employment.
Adoption of the Convention on the Rights of Persons with Disabilities, 13 December 2006	The CRPD was applied by the United Nations General Assembly, establishing articles addressing the right of equal employment as an international standard
CRPD Enters into Force, 3 May 2008	The CRPD was finally legally binding to member states with its 20th national ratification.
Adoption of the 2030 Agenda for Sustainable Development, 25 September 2015	SDGs were first adopted by the United Nations General Assembly. It calls upon equal and fair employment and encourages proper workspace qualities for people who are undergoing diverse disorders.
UN Political Declaration on HIV and AIDS: Ending Inequalities and Getting on Track to End AIDS by 2030, 8 June 2021	The United Nations General Assembly improves various international commitments to settle the social and ethnic problems related to HIV exclusively considering employment

	settings.
WHO Global Report on Health Equity for Persons with Disabilities, 2 December 2022	WHO announces their most comprehensive article addressing health and economic inequalities encountered by citizens with medical affliction, and assume that the employment gaps as a health issue.

UN Involvement, Resolutions, Treaties and Events

- ILO Convention No. 111, June 25 1958]

This convention is one of the ILO's fundamental conventions, and it requires member states to adopt national policies that promote equality of opportunity and treatment in employment and occupation. It defines discrimination as any distinction, exclusion, or preference based on race, colour, sex, religion, political opinion, national extraction, or social origin that impairs equality in employment. Notably, disability and health status are not included in this list, although the convention allows each member state to add other grounds after consulting employers' and workers' organizations. This gap shows that the main international convention on employment discrimination does not automatically protect individuals with medical conditions, which is one of the key reasons this issue requires further international action.

- UN Declaration on the Rights of Disabled Persons, December 9 1975

This declaration was the first international measure to confirm that people with medical conditions should be treated as an equal civil and ethical figure just like other people. This declaration also stated the standards of assistive measures that help disabled people become self-reliant. It also affirmed that people who are enduring diverse ailments have the right to pursue economic and social benefits and stability, and to have high-quality of living. Additionally, they also have the right to secure their place in the workforce. This document was not legally binding yet it played a role as a foundation for future practices for disabled people.

- ILO Convention No. 159, June 20 1983

This convention requires nations to establish and implement national regulations about vocational training and employment for people with medical issues. The policy should follow the basic standard of equal opportunity of job applicants. These national laws also should include employment ensuring frameworks such as special positive systems without being considered as another discriminatory for most workers. This convention is considered

as an important foundation for numerous nations' legal frameworks such as France, China and Germany.

- UN General Assembly Declaration of Commitment on HIV/AIDS, June 27 2001

This document was adopted during the UN General Assembly's first Special Session addressing global health issues, recognizing the severe problems caused by HIV and AIDS. These kinds of problems include stigma, social exclusion, and employment discrimination. This declaration has called upon the state governments to implement laws to safeguard people living with HIV and AIDS' daily lives from discrimination in the workplace. The goal was to reduce the degree of workforce inequity, for instance, hindering promotion, cooperation and individual salaries.

- Convention on the Rights of Persons with Disabilities, December 13 2006

The CRPD is the first ever legally bound human right treaty which was targeted for people who are undergoing medical conditions. By accepting the social definition of disabilities, it has widened the understanding of various kinds of impairments and that they are also caused by social barriers, as well as inherent problems. This convention calls on nations to be aware of the right of humans with disabilities to work in unison with most workers. It has been ratified by 186 countries, making it one of the most widely recognized human right conventions in the UN.

- 2030 Agenda for Sustainable Development, September 25 2015

The 2030 Agenda established 17 Sustainable Development Goals (SDGs), which all UN member states agreed to pursue by 2030. Target 8.5 calls for full and productive employment and decent work for all women and men, including persons with disabilities, and equal pay for work of equal value. Target 10.2 aims to promote the social and economic inclusion of all people, regardless of disability, and Target 3.4 aims to reduce premature deaths from non-communicable diseases by one third. By linking health, employment, and equality, the 2030 Agenda provides a framework that connects the main concerns of this committee.

- ILO Global Survey on HIV Discrimination in the World of Work, 2021

  This survey, conducted by the International Labour Organization together with Gallup, collected responses from more than 55,000 people in 50 countries to understand why HIV-related stigma and discrimination persist in the workplace. It found that only about one in two people knew that HIV cannot be transmitted by sharing a bathroom, and that as many as six in ten people supported mandatory HIV testing before people are allowed to work. The survey concluded that a lack of basic knowledge about HIV transmission is a major cause of stigma, and it called for renewed HIV education programs in the world of work.

- Political Declaration on HIV and AIDS: Ending Inequalities and Getting on Track to End AIDS by 2030, June 8 2021

  This declaration was adopted at the UN General Assembly High-Level Meeting on HIV and AIDS, and it renewed international commitments to end AIDS as a public health threat by 2030. It recognized that inequalities, rather than the virus alone, continue to drive the HIV epidemic. Member states committed to reducing HIV-related stigma and discrimination, including in workplace settings, and to removing laws and policies that unfairly target people living with HIV. The declaration shows that discrimination is now widely recognized as a barrier not only to employment but also to public health.

- WHO World Mental Health Report: Transforming Mental Health for All, June 17 2022

  This report is WHO's largest review of global mental health since the start of the century. It found that nearly one billion people worldwide were living with a mental disorder in 2019, and it highlighted that stigma, discrimination, and human rights violations against people with mental health conditions remain widespread, including in employment. The report called on all countries to transform mental health by deepening their commitment to it, reshaping the environments that affect mental health, such as workplaces, and strengthening community-based care.

- WHO Global Report on Health Equity for Persons with Disabilities, December 2 2022

This report, which was published in a decade after the WHO announced the World Report on Disability, shows that there are still citizens with medical problems who face prejudice. It has presented that those people are encountering unequal situations in welfare, education, and employment. This report explicitly focuses on the accessibility of employment, and the assistive technologies that would help disabled people improve their self-reliant abilities. WHO, as a major organization of healthcare and equality, encourages all member states to enhance their legal frameworks of health and labour administration within their nation.

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